

HOW TO OPEN A BRAIN INJURY PERSONAL CARE HOME in Georgia

A DCH-licensed Personal Care Home — funded through Georgia's ICWP and CCSP/SOURCE waivers.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Georgia model — read this first

In Georgia, a home that provides housing, meals, and personal care to adults with brain injury is licensed by the Georgia Department of Community Health (DCH), Healthcare Facility Regulation Division (HFRD), as a Personal Care Home (PCH) under Rules Chapter 111-8-62. A PCH is a needs-based license — not age- or diagnosis-based — so brain-injury adults are served under the same PCH license. Georgia is unusual in a good way: it has a Medicaid waiver built specifically for adults with traumatic brain injury (the Independent Care Waiver Program), and a state Brain & Spinal Injury Trust Fund. This guide walks the whole path — licensing the home, then getting paid — broken down so a first-timer can follow it.

PCH vs. ALC — pick the right size and acuity. Georgia splits residential care by size and how much you do. A Personal Care Home (under 25 residents) provides housing, food, and personal services — and in a plain PCH staff may only ASSIST with self-administered medication. An Assisted Living Community (ALC, 25+ residents, Chapter 111-8-63) can administer medications with certified medication aides and provide limited nursing. Most brain-injury homes start as a PCH; the key question is medications (see Part 5). If your residents can't self-administer, you'll staff a nurse or trained proxy caregiver, add a Memory Care Center certificate, or license as an ALC.

Accreditation — strongly recommended, and how we build. Georgia does not require national accreditation to hold a PCH license, but for a brain-injury program it is a real advantage: CARF (which offers a Brain Injury specialty) and The Joint Commission signal quality to case managers, hospital discharge planners (including the Shepherd Center), and families, and can strengthen your standing with payers. Every PolicyReady product is built to those standards behind the scenes. Treat accreditation as a goal from day one (see Parts 7 & 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with DCH/HFRD, Georgia Medicaid, your Area Agency on Aging, and qualified counsel (see Part 11) before you rely on it.

Before you begin — a realistic picture

Be clear-eyed about what you're starting. A brain-injury personal care home is a licensed, capital- and staffing-intensive business: you're leasing or buying a building, bringing it up to the 111-8-62 physical-plant and fire specs, staffing awake caregivers around the clock, and carrying real payroll and insurance before your first Medicaid dollar arrives. The licensing path is not instant — HFRD's review runs 60 days once your packet is complete, and assembling that packet (zoning, the fire and electrical inspections, the floor sketch, everyone's background checks) usually takes longer than the review itself. Then Medicaid enrollment and waiver enrollment add more weeks, and CCSP/SOURCE only open in the March and September windows. Plan the approval timeline generously, and hold a

working-capital cushion that covers several months of rent, payroll, insurance, and food before census and reimbursement ramp — most homes fill beds gradually, not overnight.

The journey at a glance

Ten milestones take you from idea to a licensed, enrolled, paid home. This guide follows them in order: Parts 1–2 explain what the business is and who regulates you; Parts 3–4 form the company and prepare the building; Parts 5–6 staff and license the home; Parts 7–8 move a resident through your doors and get you paid; and Parts 9–11 cover costs, referrals, and every contact to call. Each part below is one rung.

1	Decide this is your business	A licensed, 24-hour brain-injury PCH — durable, census-based revenue (Part 1)
2	Form the business & insure	Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)
3	Secure & prepare the home	Zoning, and the 111-8-62 physical-plant specs (Parts 3–4)
4	Set up your administrator	The PCH administrator/on-site manager qualifications + 16 hrs/yr CE (Part 5)
5	Build & train staff	First aid/CPR, medication handling, the staffing ratios, background checks (Part 5)
6	Write your policies	P&P Manual + the 111-8-62 operating standards (Parts 4–6)
7	Get the DCH license	Apply via HFRD; fire, electrical & pre-licensure review; license (Part 6)
8	Enroll for Medicaid waivers	Georgia Medicaid (GAMMIS) + ICWP and/or CCSP/SOURCE (Part 8)
9	Get your first residents	Shepherd Center / hospital discharge planners + AAAs (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Start your fire-safety and electrical inspections and your background checks early — the application won't move without them (the fire report must be dated within 12 months, the electrical within 6). Do the cheap, reversible steps (form the entity, sign a contingent lease) first.

PART 1

What This Business Is

In short: A DCH-licensed Personal Care Home for brain-injury adults — housing, food, and personal services (help with medications, mobility, and daily living). It's a real-estate + 24-hour-care business.

Georgia law defines a Personal Care Home as a dwelling that provides housing, food service, and one or more personal services to two or more unrelated adults. "Personal services" means individual assistance with or supervision of self-administered medication, help with ambulation and transfer, and help with the essential activities of daily living — eating, bathing, grooming, dressing, and toileting. It's the assisted-living-type license — you're licensed for the level of care your residents need, not for a diagnosis — so a home built for adults with brain injury operates under the same PCH license as any other.

Size & acuity — where a PCH sits

Setting	Size	What it can do
Personal Care Home (PCH)	2-24 residents	Housing, food, personal services; staff ASSIST with self-administered medication
Memory Care Center (add-on)	a certified secured unit	Serve cognitively impaired residents in a secured setting (extra staff & drill rules)
Assisted Living Community (ALC)	25+ residents	Adds medication administration by certified med aides and limited nursing

Serving residents who wander

If a resident is prone to wandering or elopement (common after brain injury) and you'll secure/lock the setting, Georgia requires a Memory Care Center certificate — no secured memory-care unit may operate, and no such residents may be admitted to it, without that current certificate (111-8-62-.05(11)). A memory care center carries extra staffing and fire-drill rules (including evacuating all residents within 13 minutes where residents are non-ambulatory). Decide early whether you'll run a secured setting.

Key terms. DCH = Georgia Dept. of Community Health (your licensor). HFRD = Healthcare Facility Regulation Division. PCH = Personal Care Home. ALC = Assisted Living Community. ICWP = Independent Care Waiver Program (the adult-TBI Medicaid waiver). CCSP/SOURCE = the Elderly & Disabled Waiver programs that pay for Alternative Living Services in a PCH. ALS = Alternative Living Services (the residential waiver service). AAA/ADRC = Area Agency on Aging / Aging & Disability Resource Connection (assessment & referral hub). BSITF = Brain & Spinal Injury Trust Fund. GCHEXS/GAPS = the fingerprint background-check system. CMA = Certified Medication Aide. GAMMIS = Georgia's Medicaid enrollment/claims system. NPI = National Provider Identifier. CARF/TJC = the two accreditors.

PART 2

Who Regulates You (and What Each One Does)

In short: DCH/HFRD licenses you, the fire authority clears your building, Georgia Medicaid + your Area Agency on Aging pay you and send residents, and the Long-Term Care Ombudsman advocates for residents.

DCH / HFRD — your licensor

The Georgia Department of Community Health, Healthcare Facility Regulation Division, issues your PCH license, writes the 111-8-62 standards, reviews your application and floor plans, and conducts the pre-licensure and ongoing (roughly every 11–15 months) inspections. Personal Care Home program: (404) 656-4507; dch.georgia.gov (HFRD).

The fire authority (Safety Fire Commissioner / local fire marshal)

Before DCH licenses you, the appropriate fire-safety authority (the Georgia Office of Insurance and Safety Fire Commissioner and/or your local fire marshal) inspects the building against the fire code and NFPA 101. Your fire-safety inspection report (no violations, showing capacity) must be dated within 12 months of your application. For a brain-injury home whose residents may not self-evacuate, engage them early — especially if you'll run a secured memory-care unit.

Georgia Medicaid + your Area Agency on Aging (who pay you / send residents)

Georgia Medicaid (DCH) runs the waivers that pay for residential care — the Independent Care Waiver Program (ICWP, for adults with TBI/physical disability) and the CCSP/SOURCE Elderly & Disabled programs (Alternative Living Services in a PCH). Access for CCSP/SOURCE runs through your regional Area Agency on Aging (AAA) / Aging & Disability Resource Connection, which assesses clients and places them — making the AAA a key referral source. Statewide Aging/ADRC line 1-866-552-4464.

Long-Term Care Ombudsman

The Georgia State Long-Term Care Ombudsman is a free, independent advocate for residents of personal care homes, assisted living, and nursing facilities — you post the number for residents (1-866-552-4464; georgiaombudsman.org). You must report suspected abuse, neglect, or exploitation.

PART 3

Form Your Business & Secure the Home — Step by Step

In short: *This is where people get stuck. We break each piece down: what it is, where to go, and exactly what to do.*

3.1 Form your legal entity

An “entity” is the legal business (usually an LLC) that holds the license and bank account and shields your personal assets. In Georgia you form it at the Secretary of State.

1. **Go to the Georgia Secretary of State.** File online through the SOS eCorp system (ecorp.sos.ga.gov; Corporations Division 844-753-7825).
2. **File Articles of Organization (LLC).** The filing fee is \$100 online (\$110 by mail). Name the LLC and list a registered agent (receives legal mail — can be you).
3. **Adopt an Operating Agreement** — the internal document setting out ownership, management, and money. Georgia doesn't file it, but your bank and partners will want it.
4. **Calendar your annual registration.** Every Georgia entity files an annual registration between January 1 and April 1 — \$50 (or \$60 filed online) — first due the year after you form.

A Georgia publication nuance. *Forming an LLC does NOT require newspaper publication. But if you operate under a trade name / DBA, Georgia registers that at the COUNTY level (Clerk of Superior Court) and requires you to publish the trade-name registration in the county's legal newspaper. Plan for that only if your public name differs from the LLC's legal name.*

3.2 Get your EIN (federal tax ID)

An EIN is your business's federal tax number — free from the IRS, and you need it for the bank, payroll, and Medicaid enrollment.

1. **Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
2. **Apply online** with the responsible party's name and SSN/ITIN and your entity info.
3. **Save the confirmation (CP-575)** — you get the EIN immediately.

3.3 Open a business bank account

What to have ready:

- EIN confirmation from the IRS
- Your Articles of Organization and the control/entity number
- Operating Agreement (LLC)
- A Beneficial Ownership form (owners of 25%+ and a control person)
- Government photo ID, SSN, DOB, home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: a major bank with lots of Georgia branches — Truist (an Atlanta-based bank), Wells Fargo, Bank of America, Synovus, or a local credit union. Compare monthly fees and how to waive them, transaction/cash-deposit limits, branch access, online bill pay, and whether they offer a business line of credit.

3.4 Get your insurance — what you need, who sells it, and what to ask

A personal care home cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy:

Coverage	What it protects against	Must-have?
General Liability (GL)	A visitor/family slip-and-fall or property damage on your premises	Yes — foundational
Professional Liability (malpractice / E&O)	Claims about the CARE — supervision, med errors, neglect	Yes — top healthcare coverage

Abuse & Molestation	Allegations of abuse by staff or others (GL excludes this!)	Yes — high exposure
Commercial Property	The building, contents, equipment; add Business Interruption	Yes if you own; often if you lease
Commercial Auto (incl. hired/non-owned)	Transporting residents; staff driving own cars for work	Yes if you drive residents
Workers' Compensation	Staff injuries (lifting/transferring is high-risk)	Required in Georgia at 3+ employees
Cyber Liability	A breach of protected health information (HIPAA)	Strongly recommended

Georgia workers'-comp threshold. Georgia requires workers' compensation once you have three or more employees (full- or part-time). A brain-injury home will cross that quickly, so plan for it as you staff up.

Real places to get it

Buy through an independent agent/broker who accesses specialty carriers. Carriers that write senior-care/residential-care risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Crum & Forster, Selective, and Nationwide. Specialty programs/brokers for this niche include Caitlin Morgan, Gallagher, AssuredPartners, Professional Insurance Group, and XInsurance. To find a local Georgia agent, use the “Trusted Choice” Find-an-Agent directory at [trustedchoice.com](https://www.trustedchoice.com) (filter healthcare/senior-care), or ask a licensed peer. Verify each carrier writes personal care homes in Georgia before relying on a quote.

What to ask the insurance agent

“Do you write personal care homes / brain-injury homes — which carriers?”

“Is professional liability included — claims-made or occurrence — and what limits?”

“Is abuse & molestation included, at what limit, sub-limit or full?”

“Are GL and professional liability shared limits or separate?”

“What limits does a waiver contract or my landlord/lender require?”

“When do I need workers' comp, and what will it cost given my staffing?”

3.5 Secure & zone the home

- ☐ Call your city/county ZONING office: is a personal care home for [n] residents allowed at this address, or do you need a permit? Georgia group homes for people with disabilities generally have residential-use protection under fair-housing law — get the zoning approval in writing (you'll submit it with the application).
- ☐ Confirm the building can meet the 111-8-62 physical-plant specs (Part 4) and be made accessible.
- ☐ Line up your fire-safety inspection (dated within 12 months) and electrical inspection (within 6 months) — you submit both with the application.
- ☐ If leasing, make sure the lease allows your use and resident count, and make it contingent on licensure.

3.6 Map ownership + background checks

Identify every owner and controlling person — prospective owners, administrators, managers, and any employee with direct access to residents must clear a fingerprint background check through GCHEXS before the license issues (Part 5.4). Your satisfactory determination letter must be dated within 12 months of the application.

3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID payers use to identify you. Your FACILITY needs a Type 2 (Organizational) NPI:

1. **Create an I&A account** at nppes.cms.hhs.gov (Identity & Access) for the person who'll manage the number (usually an owner).
2. **Log into NPPES** and start a new application; choose Type 2 (Organizational).
3. **Enter your legal name, EIN, and address** and pick the taxonomy that fits a personal care / residential facility.

4. Submit — it's free and usually processes in about 10 business days. Save your NPI; you'll need it for Medicaid enrollment (Part 8).

***What this means for you.** Part 3 is the paperwork foundation the rest of the guide stands on — the entity holds the license, the EIN and NPI make you billable, and the zoning letter and inspections are the attachments HFRD requires. None of it is clinical, so knock it out early: much of it is free (EIN, NPI) or cheap (the \$100 LLC), and it's all reversible before you sign a lease. The single most expensive mistake at this stage is committing to a building before you've confirmed zoning and the 111-8-62 fit — make every lease contingent on licensure.*

Part 3 completion checklist

- ☐ LLC formed at the Secretary of State; Operating Agreement adopted; annual-registration date calendared
- ☐ EIN issued by the IRS (CP-575 saved)
- ☐ Business bank account opened and funded; owners/signers on file
- ☐ Insurance quotes in hand — GL, professional, abuse & molestation, property, auto, workers' comp
- ☐ Zoning approval for a PCH at your address, in writing
- ☐ Fire (within 12 months) and electrical (within 6 months) inspections scheduled
- ☐ Every owner, administrator, manager, and direct-access employee started through GCHEXS
- ☐ Type 2 (Organizational) NPI issued and saved for Medicaid enrollment

PART 4

The Home — Physical Plant, Safety & Fire Code (111-8-62)

In short: Georgia gives you real numbers for bedrooms, bathrooms, and water, and runs fire safety through the Safety Fire Commissioner. Here's what each requirement means and how it's checked.

"Physical plant" just means the building itself and everything fixed in it — the rooms, plumbing, kitchen, and fire systems. DCH inspects the health/physical side and the fire authority inspects fire & life safety before you're licensed. The rules live in Rules Chapter 111-8-62. Below, each requirement is followed by what it means for you.

4.1 Bedrooms & space (111-8-62-.12)

- ☐ At least 80 sq ft of usable floor space per resident (70 sq ft for homes licensed before Feb 6, 1981)
- ☐ No more than 4 residents per bedroom / private living space
- ☐ Each resident has their own bed and personal storage
- ☐ A living/common area and a dining area sized for the residents

What this means for you: Georgia measures by square feet PER resident, so a room can hold more people if it's big enough — but never more than four to a bedroom. Measure every bedroom before you sign a lease, and remember brain-injury residents often use wheelchairs or walkers, so leave real turning room beyond the 80-sq-ft minimum.

4.2 Bathrooms & water (111-8-62-.12 / -.13)

- **Toilets & sinks:** at least one functional toilet and lavatory (sink) for every 4 residents, and at least one toilet and lavatory on each floor that has resident bedrooms.
- **Bathing:** at least one bathing or showering facility for every 8 residents.
- **Safety:** grab bars at toilets and in tubs/showers, non-slip surfaces, and features for residents using wheelchairs/walkers/transfer benches.
- **Hot water:** comfortable to the touch but not exceeding 120°F — capped to prevent scalds; verify with a thermometer and document it.

What this means for you: think like a surveyor walking your bathrooms and kitchen. Bathroom ratios, safe water temperature, and safe storage of chemicals and medications are the common early findings. Verify your water temperature, add grab bars, and lock up chemicals and medications before you apply.

4.3 Fire & building safety (111-8-62-.13)

- ☐ Smoke detectors powered by the house electrical service with battery backup, audible in the sleeping rooms
- ☐ At least one charged 5-lb multipurpose ABC fire extinguisher on each occupied floor and in the basement
- ☐ Compliance with the Safety Fire Commissioner's rules; PCHs are regulated as NFPA 101 Residential Board & Care occupancies
- ☐ A fire-safety inspection report with no violations, dated within 12 months, submitted with your application
- ☐ A written emergency & evacuation plan and staff trained to move residents; keep every inspection report in one dated compliance binder

Confirm your sprinkler requirement early. Whether an automatic sprinkler system is required depends on your building's size, number of stories, and your residents' ability to evacuate. For a brain-injury or memory-care home, evacuation capability is central — confirm the exact sprinkler and alarm requirements with the State Fire Marshal / local fire authority for your specific building before you commit or renovate. A memory-care unit with non-ambulatory residents must be able to evacuate everyone within 13 minutes.

4.4 Brain-injury environmental design (why it matters here)

Beyond the code minimums, design for the residents' specific risks: clear, low-clutter walking paths to reduce falls; consistent, labeled locations for personal items (memory and orientation support); a code-compliant secured

setting or door alarms if any resident wanders (with the Memory Care Center certificate if you lock exits); reduced noise and visual overstimulation in common areas; and a quiet space a dysregulated resident can use. This is both good care and part of keeping residents safe.

4.5 Walk your building like a surveyor — a pre-inspection checklist

Before you invite anyone official in, walk every room with this list and a thermometer. Fixing these yourself is far cheaper than a re-inspection or a licensing delay, and most early findings come straight off this page.

- ☐ Measure each bedroom: at least 80 sq ft of usable floor per resident, no more than 4 to a room, a bed and lockable storage each
- ☐ Count fixtures: one toilet + sink per 4 residents, one bathing/shower per 8, a toilet and sink on every floor with bedrooms
- ☐ Test hot water at the tap: comfortable but not over 120°F — record the reading and the date
- ☐ Grab bars at every toilet and in every tub/shower; non-slip surfaces; clear wheelchair/walker turning room
- ☐ Smoke detectors hard-wired with battery backup and audible in each sleeping room; test them
- ☐ A charged 5-lb ABC extinguisher on every occupied floor and in the basement, tag current
- ☐ All medications and chemicals/cleaners locked and separated from food
- ☐ Exits clear and unobstructed; written emergency & evacuation plan posted; staff know the routes
- ☐ One dated compliance binder holding the fire report, electrical form, and every inspection

What this means for you. *The three findings that most often surface first are bathroom-to-resident ratios, water temperature over 120°F, and unlocked medications or chemicals. Clear those before you apply and your pre-licensure visit goes far more smoothly.*

PART 5

Staff, the Administrator & Required Training — by Position

In short: Georgia is specific about who's on duty and about medications. Here it is, one by one — and the medication rule is the piece that shapes your staffing in a brain-injury home.

5.1 Your administrator / on-site manager (111-8-62-.08)

Every PCH must have an administrator or on-site manager (who may be you). The requirement scales with size:

- Homes with 24 or fewer beds.** The administrator/on-site manager must be at least 21 and hold EITHER an associate's degree OR a high-school diploma/GED plus two years of experience in a licensed PCH or other healthcare-related setting.
- Homes with 25 or more beds.** The administrator must hold a valid license from the Georgia State Board of Long-Term Care Facility Administrators.
- Coverage.** At least one administrator, on-site manager, or responsible staff person must be on the premises 24 hours a day.
- Continuing education.** The administrator — like all direct-care staff — completes at least 16 hours of continuing education each year, and new staff finish job training within the first 60 days.

Good news for a small home. For a home under 25 beds you do NOT need a state-licensed nursing-home administrator — an associate's degree, or a high-school diploma plus two years of relevant experience, qualifies you to run it. Confirm your pathway against 111-8-62-.08.

5.2 Medication — the acuity gate that shapes your staffing (111-8-62-.20)

This is the rule to build your staffing around. In a plain Personal Care Home, staff may only ASSIST residents with self-administered medication — read the label, open the container, hand over a prescribed dose — for residents who can direct it, after medication training and an annual competency demonstration. Staff may NOT actually administer medications. To administer medications, you need a licensed nurse, a Certified Medication Aide (CMA), or a trained proxy caregiver authorized under Chapter 111-8-100. Because brain-injury residents often cannot safely self-administer, plan from day one either to employ a nurse or trained proxy caregivers, or to add a Memory Care Center certificate / license as an ALC (where CMAs may administer). This decision drives your staffing and budget more than any other.

5.3 Staffing ratios & coverage (111-8-62-.10)

Georgia sets minimum awake direct-care staffing ratios — and yes, awake night staff is required:

Home size	Waking hours	Non-waking hours
Under 25 beds	1 awake direct-care staff per 15 residents	1 awake direct-care staff per 25 residents
25 or more beds	1 awake direct-care staff per 15 residents	1 awake direct-care staff per 20 residents
Memory care unit	1 dementia-trained staff per 12 residents	1 dementia-trained staff per 15 residents

There must also be at least one staff person on each occupied floor (homes of 25+ need at least two staff present at all times). A brain-injury home's acuity may require richer staffing than these minimums — staff to your residents' needs, not just the floor.

5.4 Training, brain-injury competence & background checks

- Training:** direct-care staff complete at least 16 hours of continuing education a year (medication assistance, working with the elderly, cognitive impairment, mental illness, evacuation, residents' rights, abuse reporting, infection control), plus current first-aid AND CPR certification (a course with a hands-on competency demonstration).

- **Memory-care training:** if you run a secured unit, add at least 4 hours of dementia orientation within 30 days and 16 hours of specialized training for direct-care staff, with an RN/LPN on-site 8–40 hours a week depending on census.

- **Brain-injury competence:** Georgia doesn't spell out a TBI-specific module, so build it in: cognitive stimulation, wander- and personal-safety, responding to dysregulated behavior, and communication strategies.

1. Fingerprint every required person through GCHEXS. Owners, administrators, managers, and any employee with direct access to residents must be fingerprinted (scheduled through GAPS / IdentoGO).

2. Budget the fee & timing. About \$51.50 per applicant; the satisfactory determination letter (on DCH letterhead) must be dated within 12 months of your license application. Background-check unit: (833) 463-0020.

3. Document everything. Keep the dated results in each file; a surveyor will ask to see them.

What this means for you. Staffing is your single largest ongoing cost, and the medication rule in 5.2 is what sets it. A plain PCH whose residents can direct their own medications runs leanest; the moment your residents need medications administered, you're paying for a nurse, trained proxy caregivers, or a CMA in an ALC — build that into your budget before you accept those admissions, not after.

Each direct-care staff file should hold

- ☐ GCHEXS satisfactory-determination letter (dated within 12 months of hire/application)
- ☐ Current first-aid AND CPR certification with a hands-on competency demonstration
- ☐ Medication-assistance training and the annual competency demonstration
- ☐ The 16 hours of annual continuing education, with topics and dates
- ☐ New-staff job training completed within the first 60 days
- ☐ For a memory-care unit: 4 hours of dementia orientation within 30 days plus 16 hours of specialized training
- ☐ Brain-injury competencies: cognitive stimulation, wander/personal safety, de-escalation, communication

PART 6

Get Your DCH License — Step by Step

In short: Apply to HFRD with your zoning approval, inspections, floor sketch, and background checks; pass the pre-licensure review; and receive your PCH license. The initial review period is 60 days.

6.1 Assemble the application (111-8-62-.05)

- ☐ Completed, signed application + your LLC Articles of Organization
- ☐ County/city zoning approval documentation
- ☐ Notarized Affidavit of Personal Identification + copy of ID; the Administrator & Owner Survey
- ☐ Proof of ownership or lease; water/sewage (or septic) approval
- ☐ Fire-safety inspection report (no violations, showing capacity) dated within 12 months
- ☐ Electrical inspection compliance form from a Georgia-licensed electrician dated within 6 months
- ☐ A labeled floor sketch (rooms, bedroom measurements, doors/windows, bed placement)
- ☐ GCHEXS background-check determination letter (within 12 months) for administrators, managers, and owners
- ☐ (Only for 25+ beds) a Certificate of Need determination and a notarized Affidavit of Financial Stability

6.2 Submit, pay & pass the review

- 1. Apply through HFRD.** Use the HFRD forms/online intake (dch.georgia.gov → HFRD → Forms & Applications) and the HFRD payment portal.
- 2. Pay the fees.** A \$300 initial application processing fee, plus the annual license fee by bed count — under 25 beds is \$350 (25 to <50 is \$750; over 50 is \$1,500).
- 3. Pass fire & pre-licensure review.** The fire authority's inspection and HFRD's pre-licensure review confirm the home meets the standards.
- 4. Receive your PCH license.** Then keep it current — HFRD conducts unannounced inspections roughly every 11–15 months.

6.3 Timeline

HFRD's initial review period is 60 days from the date it receives your application, but assembling the pieces — zoning, the fire and electrical inspections, the floor sketch, and everyone's background checks — is what usually sets your real start date. Line those up in parallel so your first submission is complete.

Ask before you file. Call HFRD's Personal Care Home program at (404) 656-4507 to confirm the current application packet, fees, and required attachments, and the background-check unit at (833) 463-0020 about GCHEXS — a complete first submission saves weeks.

PART 7

The Resident's Journey — Referral to Discharge

In short: A waiver client is assessed and referred (by a case manager, an AAA, or a hospital/Shepherd discharge planner); you confirm the fit, admit under a residency agreement, deliver and document the care plan, and plan for the least-restrictive setting. Here is the whole journey, stage by stage.

Stage 1 — Referral comes in

A referral comes from an ICWP case manager, an Area Agency on Aging (for CCSP/SOURCE), or a hospital or rehab (e.g., Shepherd Center, Emory Rehabilitation) discharge planner (Part 10). Someone asks whether you have a bed for a specific person.

What you do:

- Take the basics (age, diagnosis, waiver, level of care, funding, target move date). Ask for the person's assessment and service plan. Tell them your license, whether you offer a secured memory-care unit, and your current openings.

Stage 2 — Pre-admission screening (can you safely serve this person?)

Before you say yes, confirm the resident fits a PCH, your capacity, and your abilities. A PCH provides personal services — if the person needs routine medication administration, be sure your nurse/proxy-caregiver plan covers it; if they wander, be sure your setting and certificate cover it.

What to review:

- Diagnoses and behaviors (aggression, elopement/wandering, swallowing/choking risk); mobility and transfer needs; medications and who will administer them; and whether your trained staffing can meet all of it safely. When in doubt, ask the case manager and the resident's physician.

Stage 3 — The case manager / AAA sets the plan

For waiver clients, the ICWP case manager or the AAA — not you — determines eligibility and level of care, confirms financial eligibility, and builds the service plan that authorizes Alternative Living Services in your home. The member chooses among enrolled providers.

Stage 4 — Admission & the residency agreement

On move-in, the resident (or their representative) signs a residency agreement covering services, resident rights, fees, who pays room and board, and how residency can end. Give a rights orientation and open a resident record.

The resident record must include:

- Health information and diagnoses; a current medication list with allergies; advance directives / power of attorney; physician and case-manager contacts; emergency contacts and the representative; and the signed agreement and consents.

Stage 5 — Build the care plan (the roadmap)

Within your policy timeframe, develop and follow the resident's care plan — the day-to-day plan for THIS person, aligned to the waiver service plan. For a brain-injury resident it typically covers cognitive-stimulation activities, personal- and wander-safety measures, help with the activities of daily living, medication handling by the appropriate staff, nutrition and any special diet, and behavior supports. Coordinate with the physician and case manager, and have the resident/representative sign off.

Stage 6 — Daily life & care

Deliver the plan: structured daily routines, ADL support, cognitive and social activities, community outings as appropriate, meals, and safe medication practices — all mapped to the resident's goals. Maintain your required awake coverage and floor staffing, and document progress notes so you can show the plan is being followed.

Stage 7 — Reviews, incidents & rights

Review and update the care plan as needs change and whenever the case manager reassesses. Run your quality program with incident reports; protect resident rights; and report any suspected abuse, neglect, or exploitation immediately. Everything is documented and followed up.

Stage 8 — Discharge & transition

When a resident's needs change or goals are met, coordinate a safe transition with the case manager and family — to a less-restrictive setting, a higher level of care (a nursing facility if they can no longer be safely served in a PCH), or the community — following your termination-of-residency policy and giving proper written notice. Send the receiving provider the records they need, and document the transition.

***What this means for you.** Every stage above leaves a paper trail, and that trail is what a surveyor reviews and what a payer audits. Two habits protect you: screen honestly at Stage 2 — never admit a resident whose acuity your staffing and certificate can't safely cover — and keep the resident record current at every stage, because a missing care-plan signature or an unlogged incident is a far more common citation than a care failure itself.*

Admission-day readiness checklist

- ☐ Pre-admission screening documented — acuity, wandering/elopement, medications, and who administers them all confirmed against your capability
- ☐ Waiver enrollment and the authorized service verified before move-in (Part 8)
- ☐ Signed residency agreement covering services, rights, fees, and how residency can end
- ☐ Resident record opened: diagnoses, current medication list with allergies, advance directives/POA, physician and case-manager contacts, emergency contacts
- ☐ Rights orientation given and documented
- ☐ Care plan started within your policy timeframe and aligned to the waiver service plan
- ☐ Room-and-board charge and monthly due date set in writing (Medicaid never pays room & board)

PART 8

How You Get Paid — ICWP & CCSP/SOURCE, Step by Step

In short: License your PCH, enroll as a Georgia Medicaid provider, and enroll in the waiver(s): ICWP for adults with TBI, and CCSP/SOURCE for elderly/disabled adults. Medicaid pays for the service — never room & board. Verify each resident's waiver before admission.

Summary of the sources:

Source	What it is	Key fact
ICWP	Independent Care Waiver Program	The adult-TBI route: ages 21–64, hospital/NF level of care; has a residential Alternative Living Services component
CCSP / SOURCE	Elderly & Disabled Waiver programs	Pay Alternative Living Services in a licensed PCH; accessed through the AAAs
BSITF	Brain & Spinal Injury Trust Fund	A state grant fund residents/families can tap for TBI/SCI services & equipment
Private pay / VA	Family self-pay; VA Aid & Attendance	Cover room & board and non-waiver care; VA may help a veteran

8.1 Which waiver funds a brain-injury resident

Georgia has no generic “assisted living” Medicaid benefit, but it does have a TBI-specific waiver — a real advantage. The Independent Care Waiver Program (ICWP) serves adults 21–64 with a severe physical impairment and/or traumatic brain injury who need hospital or nursing-facility level of care (TBI applicants are even exempt from the program's self-management requirement), and it has a residential Alternative Living Services component. For residents 65+ (or under 65 and functionally impaired) at nursing-facility level of care, the CCSP/SOURCE programs pay Alternative Living Services delivered in a licensed personal care home. Adult-onset brain injury generally does NOT qualify for the DODD NOW/COMP waivers, which require onset before age 18/22. So your two Medicaid routes are ICWP and CCSP/SOURCE.

There's a waiting list — and a grant fund. ICWP operates a waiting list, so families should apply early (intake via Alliant Health Solutions, 888-669-7195). Separately, Georgia's Brain & Spinal Injury Trust Fund (BSITF) awards grants to individuals with TBI/SCI for post-acute services and equipment — a funding source your residents and families can tap, and a resource-facilitation partner. BSITF: (888) 233-5760; bsitf.georgia.gov.

8.2 Enroll as a provider

- 1. License the PCH first (Part 6).** Some waiver enrollments expect your DCH license to have been held for several months — plan ahead.
- 2. Have your Type 2 NPI ready** (Part 3.7).
- 3. Enroll as a Georgia Medicaid provider.** Apply through GAMMIS (mmis.georgia.gov), operated by Gainwell — provide your NPI, tax info, license, and ownership disclosures. Provider Contact Center 1-800-766-4456.
- 4. Enroll in the waiver(s).** For CCSP/SOURCE, apply in the March or September windows and complete a provider consultation with your regional Area Agency on Aging. For ICWP, enroll through DCH / Alliant Health Solutions (888-669-7195).

8.3 The rate & what's covered

Alternative Living Services is paid as a per-diem (daily) rate to the enrolled personal care home; the current rate schedules live in the Medicaid manuals' appendices on GAMMIS (pull the current figure there rather than relying on a quoted number, as it changes). The waiver service plan the case manager authorizes drives what you bill.

Room & board is NOT covered by Medicaid. The waiver pays only the service — Georgia Medicaid does not cover room and board in a personal care home. The resident pays room and board from their own income (Social Security/SSI). Set a clear written room-and-board charge and a monthly due date, and build it into your admission

and residency agreement. Confirm any state-supplement figure with DCH / the Division of Aging before you rely on it.

How you actually bill (the claims cycle)

1. **Verify eligibility & the authorization** — confirm the member's waiver enrollment and the authorized service before and during the stay.
2. **Submit the claim** — the per-diem through GAMMIS, using your NPI and the authorized service.
3. **Watch timely filing** — Georgia Medicaid generally requires claims within 6 months of the date of service, with adjustments within 365 days; confirm the current rule.
4. **Collect room & board** from the resident by your written due date, separate from the Medicaid claim.

Ask DCH / your AAA / Alliant:

- Which waiver and service this resident is authorized for; the current ALS per-diem; how a reassessment changes the plan and rate; the room-and-board handling; and the claims and timely-filing details.

8.4 Private pay & veterans

- **Private pay:** non-waiver residents (and every resident's room & board) pay privately — Georgia assisted living/PCH commonly runs around \$4,900–\$5,000/month, and specialized/memory care higher; set a clear rate and a written residency/financial agreement, reviewed by counsel.
- **Veterans:** VA Aid & Attendance (an add-on to the VA pension) can help fund a veteran's care — 2025–2026 maximums are up to about \$2,424/month for a single veteran and about \$1,558/month for a surviving spouse (verify on VA.gov). VA Community Residential Care and the Polytrauma/TBI System of Care are also resources for veterans with brain injury.

PART 9

What It Costs & How Long It Takes

In short: Budget the home, the administrator and staff training, nursing/proxy coverage for medications, background checks, and the DCH fees — and consider accreditation. Plan a few months through licensing, then enrollment.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
The home	Lease deposit + first month (or purchase); accessibility mods; furnishings; life-safety systems	Varies by market/size
Medication coverage	Nurse or trained proxy caregivers (the PCH rule)	Ongoing payroll
Staff training	First aid/CPR, medication, dementia (if memory care) per staff	Per person
Background checks	GCHXS fingerprints ~\$51.50 each	Per person
Insurance	GL + professional + abuse + property + auto + workers' comp (3+ staff)	Get quotes
DCH fees	\$300 application + annual license (\$350 under 25 beds)	≈ \$650 first year (small home)
Business setup	SOS \$100 + \$50/yr registration, EIN (free), NPI (free), bank, legal	~\$150 + legal
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional but recommended)	CARF or Joint Commission (see 9.2)	Low-to-mid five figures over the cycle

9.2 If you pursue accreditation (recommended for brain injury)

- **CARF:** its Brain Injury specialty accredits residential brain-injury programs under the Medical Rehabilitation standards, and requires you to be operating and serving clients before survey. Commonly cited fees are a ~\$995 application fee plus a variable surveyor-day survey fee — request a current quote. (888) 281-6531, carf.org.
- **The Joint Commission:** offers Behavioral Health Care & Human Services and Nursing Care Center accreditation; fees are quote-based on your services and census, on a 3-year cycle. Request a quote: (630) 792-5000, jointcommission.org.
- **It's staged:** both require you to be operating before survey, then an initial on-site survey, then a decision (CARF terms run 3-year, 1-year, or provisional). Plan roughly 12–18 months. The VA's brain-injury programs are CARF-accredited, which tells you what payers value.

9.3 Timeline

Early	Inspections & background checks	Line up fire (within 12 mo) & electrical (within 6 mo); start GCHXS fingerprints
Weeks	Business & home ready	Entity, insurance, home to 111-8-62 specs, zoning approval in hand
Weeks	Staff & policies ready	Administrator, medication coverage, first aid/CPR, P&P adopted
~60 days	HFRD application → review → license	Complete application → fire + pre-licensure review → PCH license
Then	Enroll for Medicaid	GAMMIS + ICWP (Alliant) and/or CCSP/SOURCE (AAA windows)
Ongoing	Referrals & full census	Shepherd/hospital discharge planners + AAAs + BIAG

PART 10

Getting Your First Residents

In short: Residents reach you through hospital and rehab discharge planners — the Shepherd Center is Georgia's marquee brain-injury pipeline — and through the AAAs and case managers. Build those relationships before you open.

10.1 The referral sources

- **The Shepherd Center & hospital discharge planners:** the biggest pipeline for brain injury — Shepherd (the nationally-ranked Atlanta catastrophic-care rehab) and the Level 1 trauma centers discharge patients who need a residential home. Build the relationship with their case-management/social-work teams; leave a one-pager and ask how to get on their placement list.
- **Area Agencies on Aging (AAAs):** for CCSP/SOURCE, the regional AAA / Aging & Disability Resource Connection assesses clients and places them — once enrolled, stay in close contact with each AAA's intake and care-coordination staff. Statewide line 1-866-552-4464; metro-Atlanta Empowerline (404) 463-3333.
- **Brain Injury Association of Georgia (BIAG) & BSITF:** BIAG's Resource Facilitation Program connects survivors and families to providers ((470) 834-4426; braininjurygeorgia.org), and the state Brain & Spinal Injury Trust Fund coordinates resources and grants ((888) 233-5760; bsitf.georgia.gov). Two-way referral relationships worth building early.
- **Georgia 2-1-1:** the statewide information & referral line (dial 211) routes families to residential care — a no-cost inbound channel to be listed in.

10.2 The Shepherd Center & trauma / neuro-rehab centers (verify before relying)

Metro	Program (referral source)	Contact (verify)
Atlanta	Shepherd Center — the marquee GA brain/SCI rehab	(404) 352-2020; Admissions/Access (404) 350-7367
Atlanta	Grady Memorial (Level 1 / Marcus Trauma Center)	(404) 616-1000
Atlanta	Emory Rehabilitation Hospital (brain-injury cognitive rehab)	(404) 712-5512
Marietta	Wellstar Kennestone (Level 1 trauma, north metro)	wellstar.org (verify)
Augusta	Wellstar MCG Health Medical Center (Level 1)	(706) 721-0211
Savannah	Memorial Health University Medical Center (Level 1)	(912) 350-8000
Macon	Atrium Health Navicent, The Medical Center (Level 1)	(478) 633-1000

Ask each referral source:

- How do you decide where to place brain-injury patients who need a residential home? What do you need from me to be on your placement list (license, capability, availability)? Who is the discharge planner / social worker / case manager I should stay in touch with?

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & the building

Who	Contact	What to verify / ask
DCH / HFRD — Personal Care Home program	(404) 656-4507 · dch.georgia.gov (HFRD)	The application packet & fees; the 60-day review; the pre-licensure inspection
DCH Background-Check Unit (GCHEXS)	(833) 463-0020 · bgu@dch.ga.gov	The fingerprint process (GAPS); the \$51.50 fee; the 12-month letter
State Fire Marshal / local fire authority	oci.georgia.gov (Safety Fire) · your local marshal	The fire inspection; NFPA 101; sprinkler requirement; memory-care evacuation
GA Board of LTC Facility Administrators	sos.ga.gov (Professional Licensing)	Administrator licensing (only if 25+ beds)
City/County Zoning	your jurisdiction	Allowed use at your address; the written zoning approval you must submit

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Georgia Secretary of State	844-753-7825 · ecorp.sos.ga.gov	Forming your LLC (\$100); the \$50 annual registration; trade name (county)
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (Trusted Choice)	trustedchoice.com; carriers in Part 3.4	Coverages & limits; abuse & molestation; workers' comp at 3+ staff
Business bank	Truist/Wells/BofA/Synovus; a local credit union	Account fees, limits, business line of credit

Funding & referrals

Who	Contact	What to verify / ask
ICWP (via Alliant Health Solutions)	888-669-7195 · medicaid.georgia.gov	The adult-TBI waiver; the residential ALS component; the waiting list; provider enrollment
Georgia Medicaid — provider enrollment	1-800-766-4456 · mmis.georgia.gov (GAMMIS)	Enrolling your NPI; the current ALS per-diem; billing & timely filing
Area Agency on Aging / ADRC (CCSP/SOURCE)	1-866-552-4464 · aging.georgia.gov	The March/September provider windows; the AAA consultation; how referrals work
Brain & Spinal Injury Trust Fund (BSITF)	(888) 233-5760 · bsitf.georgia.gov	Grants your residents can tap; resource facilitation
Accreditor — CARF / TJC	CARF (888) 281-6531 · TJC (630) 792-5000	Cost quote; the operating-first requirement; Brain Injury specialty

Advocacy, the state BI orgs & referral partners

Who	Contact	Role
Georgia State Long-Term Care Ombudsman	1-866-552-4464 · georgiaombudsman.org	Independent resident-rights advocate — post the local number for residents
Adult Protective Services (DHS)	1-866-552-4464	Mandatory abuse/neglect/exploitation reporting
Brain Injury Association of Georgia	(470) 834-4426 · braininjurygeorgia.org	Resource Facilitation Program; survivor/family support; two-way referrals
Shepherd Center	(404) 352-2020 · shepherd.org	The flagship brain/SCI rehab — your key discharge-planning relationship
Georgia 2-1-1	dial 211 · unitedwayga.org	Statewide information & referral — a no-cost inbound channel

You can do this

Opening a Georgia brain-injury personal care home is a real build — a licensed PCH to 111-8-62 specs, a qualified administrator, trained staff and a medication plan, and vulnerable residents — but it's a defined path, and Georgia is one of the few states with a Medicaid waiver built for adults with brain injury. Take it one rung at a time: line up your inspections and background checks, form the business, prepare the home and team, get licensed by DCH, enroll for ICWP and CCSP/SOURCE, and earn referrals from the Shepherd Center, the trauma centers, and the Area Agencies on Aging. Verify each rate and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Ga. Comp. R. & Regs. Chapter 111-8-62 and current Georgia Medicaid (ICWP, CCSP/SOURCE) information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with DCH/HFRD, Georgia Medicaid, Alliant, your Area Agency on Aging, and qualified counsel before you rely on it. Confirm the current ALS per-diem and the room-and-board handling before you admit and bill.